

BUS 395 — VENTURE CREATION WITH AI — ROLLINS COLLEGE

Customer Discovery Synthesis and Business Model Canvas

Fanny Polanco

Student

Summer 2026

Term

5 Interviews

Discovery sample

9 Blocks

Canvas complete

CONTENTS

1. Venture Overview
2. Interview Synthesis — Five Profiles
3. Cross-Interview Pattern Analysis
4. Assumption Scorecard
5. Business Model Canvas
6. Remaining Risks and Next Steps

SECTION 01

Venture Overview

Refined Problem Statement

Pre-diabetic adults managing work, school, and family caregiving have enough awareness that certain foods worsen their condition but no practical system to act on that awareness consistently. Every week they buy produce with good intentions that spoils before it is used, shop without a list, spend more than they planned, and default to meals the family will accept rather than meals safe for their blood sugar. The barrier is not motivation — every interviewee

demonstrated intent. It is the absence of a system that connects grocery budget, weekly schedule, family food preferences, and blood sugar logic into something repeatable that does not require starting from scratch each week.

What changed from the original framing: The original problem statement claimed “the problem is not a lack of nutritional knowledge.” The interviews do not support that claim. Mary did not know whether oatmeal was safe for her blood sugar. Grace knew ramen was bad and bought bread and rice on the same trip without recognizing the same glycemic problem. A knowledge gap is real and coexists with the system gap. The refined statement acknowledges both without making the product responsible for solving both.

Target Customer

A pre-diabetic adult, 35 or older, who has been told by a doctor they are pre-diabetic or at high risk of Type 2 diabetes. They work full time, are a primary caregiver for a family, and operate on a grocery budget tight enough that \$60 is a ceiling, not a preference. They know one or two foods that affect blood sugar — rice, ramen, “carbs” — but cannot apply that logic consistently. They have tried at least one structured resource and stopped using it. They are not looking for more information. They are looking for something that works on a Tuesday night when they are exhausted.

Evidence base: Tere, Grace, and Mary from the five interviews. Nelsy (high blood pressure, not pre-diabetic) and Veronica (no blood sugar framework, dislikes vegetables) are excluded from the primary profile. The evidence base is three interviews — directionally consistent, not statistically validated.

Competitive Landscape

Competitor	What they built	Why they fail this customer
PlateJoy 2013–2025 • \$12.3M raised	Personalized meal planning + CDC-recognized Diabetes Prevention Program for pre-diabetics with virtual coaching.	Pivoted to B2B insurance, making revenue dependent on user health outcomes it could not control. HIPAA restricted communication. Acquired 2021, shut down 2025.
Klinio	Diabetes-labeled wellness app.	Targets clinical/insurance channel. Does not integrate budget, perishability, or schedule.
eMeals & general apps	Generic meal planning with diabetes-friendly tags.	Too rigid, not built for pre-diabetic constraints or bulk grocery reality. \$5–15/month is a structural barrier before grocery costs.
Meal kits	Pre-portioned ingredients delivered weekly.	Too expensive. Nelsy canceled after two weeks on price alone.

The gap: No product integrates all four constraints simultaneously — pre-diabetes blood sugar logic, a real grocery budget, ingredient perishability, and a family’s weekly schedule — into a repeatable, automated system. Every competitor solves one or two. None connect all four. None account for the mismatch between how food is sold (two-pound bags) and how meals are planned (one recipe).

Direction

Leaning toward direct-to-consumer AI-powered weekly food system (Remix 1): Takes a family’s budget, refrigerator inventory, local grocery sales, and weekly schedule and generates a fresh, blood-sugar-safe meal plan every week. Two core leverage points:

- **Ingredient-first planning:** Plan three meals around one bag of green beans before buying it. Breaks the waste loop at the source.
- **Built-in variety engine:** Same ingredient, different preparation each day. Breaks the fatigue loop and prevents abandonment.

B2B institutional (Remix 2) and grocery chain partnership (Remix 3) remain viable as future business model layers once the core product is validated. The “why now” argument: AI can now reason across budget, perishability, schedule, and blood sugar constraints simultaneously in a way that was not practical before.

SECTION 02

Interview Synthesis — Five Profiles

All quotes are exact. Observations below each quote set are drawn from in-transcript annotations. Challenges to assumptions are flagged separately.

Interview 1 — Tere

Pre-diabetic • Working adult • Family caregiver

“Honestly, I didn’t really ‘decide.’ I ate the same thing I usually eat — hard-boiled egg and a couple of quinoa crackers and coffee in the morning, skipped lunch because work was crazy, and then made rice, beans, and chicken for dinner because it’s what the kids will eat. I know the rice isn’t great for my sugar, but I was too tired to think of something else.”

“All the time. I buy vegetables with good intentions, but by the time I get around to cooking, they’re already soft or moldy. I feel guilty throwing them away, but it happens every week.”

“I went after work, already exhausted. I didn’t have a list. I just walked around grabbing things I usually buy. I looked at some healthier stuff, but it felt expensive, and I didn’t know what meals to make with it, so I put it back.”

“Not really. I usually just watch YouTube recipes, but they use ingredients I don’t have, and TikTok videos because the videos are shorter.”

*“Honestly? I just don’t eat or I repeat the same meal again. **I have the intention, but not a good system.**”*

CHALLENGES MY ASSUMPTIONS

She knows rice worsens her blood sugar and eats it every night — not from ignorance but because it is the only thing the kids will eat. The unit of analysis is the household, not the individual. A blood-sugar-safe plan that the children will not eat does not get followed. Produce waste produces guilt, not just financial frustration — she describes it as inevitable, not as a solvable problem. The product pitch may need to address the emotional dimension, not just the logistical one.

Interview 2 — Mary

Pre-diabetic • Working adult • Family caregiver

*“I had oatmeal for breakfast because it’s ‘healthy,’ even though **I’m not sure about the sugar.** Lunch was fast food because I was running errands. Dinner was whatever I could make quickly — Mac and Cheese and a chicken from Publix.”*

“Yes. Especially spinach and berries. They go bad so fast. I buy them because I know I should eat healthier, but I never plan meals around them.”

“I went on Sunday. I tried to buy healthier things, but I didn’t know what meals to make. I ended up buying the same stuff I always buy. I spent more than I wanted to.”

“I tried a low carb cookbook, but the recipes were too complicated. I don’t have time for all that prep.”

“TikTok. But those recipes look good and then I never make them. I think I have all the ingredients and then I’m always missing a couple of them.”

CHALLENGES MY ASSUMPTIONS

Mary directly contradicts the original claim that “the problem is not a lack of nutritional knowledge.” She does not know whether oatmeal is safe for her blood sugar. She ate Mac and Cheese for dinner with no apparent blood sugar flag — same glycemic problem as rice, not recognized. She also overspends even when not buying healthier food, suggesting the budget problem is driven by the absence of a plan, not the cost of healthy ingredients.

Interview 3 — Grace

Pre-diabetic / high risk • Budget-constrained

“I ate whatever was cheap. Eggs for breakfast, ramen for lunch, and a frozen meal for dinner. I know ramen is bad for sugar, but it’s cheap and fast — and then I try to go walking... hahah”

“Yes. I buy veggies thinking I’ll cook, but then I’m too tired after work. They sit in the fridge until they’re gross, or I cook them all at once to eat over a couple of days and they get soggy.”

“I went with a strict budget — \$60. I bought basics: eggs, bread, chicken, rice. I wanted to buy healthier stuff, but it didn’t fit the budget. I did buy mixed frozen vegetables.”

“I tried a diabetes app once, but it felt like it was made for older people. Not helpful.”

“Google. But it gives too many answers and I don’t know which one to look at. I feel like I spend a lot of time.”

CHALLENGES MY ASSUMPTIONS

Grace knows ramen is bad for blood sugar and on the same trip bought bread and rice without flagging the same glycemic problem. The knowledge does not generalize across similar foods. She also tried batch cooking as a self-developed workaround — and it failed on texture. The product needs to account for palatability across multiple days, not just planning. Her \$60 budget is a hard ceiling: any system that increases the upfront grocery bill fails regardless of long-term savings.

Interview 4 — Nelsy

High blood pressure • NOT pre-diabetic • No professional support

Note: Nelsy is not pre-diabetic. Her dietary constraint is low sodium, not blood sugar management. Her data validates the structural failure pattern but does not confirm blood sugar logic. One of five interviewees is outside the target diagnosis.

“I try to avoid salt, but it’s hard. I ate cereal for breakfast, a sandwich for lunch, and baked chicken for dinner. I decide based on what’s fast.”

“Yes. Especially things like zucchini or herbs. I buy them for one recipe and then don’t use the rest.”

“I walked around and bought the main things we always eat. I know I need low sodium options, but I don’t know which ones. I read labels, but it takes forever.”

“I tried a meal kit once, but it was too expensive. I canceled after two weeks.”

“I call mom and she gives me these recipes. They are healthy, but I don’t end up cooking them.”

CHALLENGES MY ASSUMPTIONS

Nelsy receives trusted, personalized, healthy guidance from her mother and does not act on it. The source is trusted. The recipes are good. The advice is personalized. She still does not cook them. This is the clearest evidence in the data set that the barrier is not trust, not information quality, not source credibility — it is the gap between receiving a recommendation and completing the execution work. Trust alone does not drive behavior.

Interview 5 — Veronica

Pre-diabetic / high risk

“I had yogurt for breakfast, skipped lunch, and ordered takeout for dinner. I didn’t plan anything. I just went with whatever was easiest.”

*“Yes. I buy fruit and veggies, but I forget they’re there. I don’t know how to use them before they spoil. **I don’t like vegetables. I know I need to eat them though.**”*

“I walked in with an idea of what I should buy, and I bought things that ‘seemed healthy,’ but I didn’t know what meals to make. I spent too much.”

“I tried a free meal prep app, but it felt like homework. Too many notifications.”

“Pinterest. But I never actually cook the recipes.”

CHALLENGES MY ASSUMPTIONS

Veronica does not like vegetables — the only interviewee to say so directly. A planning system cannot fix palatability. She is also the strongest evidence against a price-first adoption strategy: she tried a free app and abandoned it anyway because of friction. Price is not the only barrier. Engagement mechanics that demand ongoing input create abandonment at zero cost. She is excluded from the primary target profile on two grounds: no blood sugar framework and a palatability barrier the product cannot address.

SECTION 03

Cross-Interview Pattern Analysis

Confirmed Patterns — three or more interviewees independently

CONFIRMED — 5 OF 5

Weekly produce waste is universal and accepted as inevitable

All five interviewees confirmed weekly produce waste — not as a problem they are trying to solve, but as a recurring fact of life. Causes vary (exhaustion, forgetting, single-recipe purchasing, missing a meal plan) but the outcome is identical. This is the most consistently confirmed finding in the data.

“I feel guilty throwing them away, but it happens every week.”

— Tere

“I buy them because I know I should eat healthier, but I never plan meals around them.”

— Mary

“They sit in the fridge until they’re gross.”

— Grace

CONFIRMED — 5 OF 5

Grocery shopping happens without a list or meal plan

Every interviewee considered buying healthier food and did not, because they could not connect the item to a specific meal. The decision to put something back is not purely about cost — it is about not knowing what to do with it. This supports ingredient-first planning as a point-of-purchase intervention.

“I didn’t have a list. I just walked around grabbing things I usually buy.”

— Tere

“I bought things that ‘seemed healthy,’ but I didn’t know what meals to make.”

— Veronica

CONFIRMED — 5 OF 5

Inspiration does not convert to action — across every platform and source

TikTok, YouTube, Pinterest, Google, a trusted human being — not one source converts to a meal consistently. This is the strongest argument for the venture’s core premise: the problem is not access to food ideas. It is the bridge from idea to executed meal.

“Those recipes look good and then I never make them. I think I have all the ingredients and then I’m always missing a couple of them.”

— Mary

“I call mom and she gives me these recipes. They are healthy, but I don’t end up cooking them.”

— Nelsy

“Pinterest. But I never actually cook the recipes.”

— Veronica

CONFIRMED — 5 OF 5

Every structured resource was tried and abandoned

Five different tools. Five different failure reasons: ingredient mismatch (Tere), complexity (Mary), demographic irrelevance (Grace), price (Nelsy), friction (Veronica). The variety of failure modes matters — the problem is not fixable by improving any one dimension of an existing tool.

“The recipes were too complicated. I don’t have time for all that prep.”

— Mary (low-carb cookbook)

“It felt like it was made for older people. Not helpful.”

— Grace (diabetes app)

“It felt like homework. Too many notifications.”

— Veronica (free app)

CONFIRMED — 5 OF 5

Speed and ease override health when there is no plan

When the plan breaks down — which it does regularly — every interviewee defaults to what is fastest, not healthiest. This is a systems finding, not a motivation finding. The fix is a system that removes the decision point, not a nudge that reminds them to decide better.

“I decide based on what’s fast.”

— Nelsy

“I just went with whatever was easiest.”

— Veronica

CONFIRMED — 3 OF 5

Overspending despite not buying healthier food

Mary, Veronica, and Tere (implied) all spent more than intended without filling the cart with healthy food. Without a plan, the cart fills with familiar defaults, overspending happens anyway, and the healthier items do not make it in. The system saves money not by being cheap but by making the existing budget do what it was intended to do.

"I ended up buying the same stuff I always buy. I spent more than I wanted to."

— Mary

Contradictions

CONTRADICTION — CHALLENGES THE VENTURE DIRECTLY

They know a specific food is bad for them and eat it every day

Tere knows rice worsens her blood sugar and eats it every night. Grace knows ramen is bad and eats it for lunch. Knowledge is present. Behavior does not change. The product's answer must be that the system removes the decision entirely — the better option is prepped and accessible before the exhaustion point arrives. But that mechanism depends on the customer following the plan in advance, which is the behavior the venture is trying to produce. The circularity is real and unresolved.

"I know the rice isn't great for my sugar, but I was too tired to think of something else."

— Tere

"I know ramen is bad for sugar, but it's cheap and fast."

— Grace

CONTRADICTION — COMPLICATES THE PROBLEM STATEMENT

Grace's blood sugar knowledge does not transfer across similar foods

Grace knows ramen is bad for blood sugar. On the same trip she bought bread and rice with no apparent flag — the same glycemic problem, unrecognized. The knowledge does not generalize. A system that surfaces brief rationale cannot assume the customer will connect "bread is like ramen" without being told explicitly.

CONTRADICTION — REFRAMES THE TRUST PROBLEM

Trusted, personalized guidance from a human being does not produce action

Nelsy's mother gives her healthy, personalized recipes from a completely trusted source. She does not cook them. The failure is not in the source, quality, or credibility of the advice. It is in the gap between receiving a recommendation and completing the execution work. This reframes the AI trust question:

“will she trust an AI?” may be the wrong question. The right question is whether the output is complete enough that she does not have to do anything after receiving it except follow it.

“I call mom and she gives me these recipes. They are healthy, but I don’t end up cooking them.”

— Nelsy

CONTRADICTION — AGAINST A PRICE-FIRST STRATEGY

A free product was abandoned for reasons unrelated to price

Veronica tried a free dedicated app and abandoned it because it felt like homework and had too many notifications. Price is a structural barrier confirmed by the competitive analysis and three interviews. But removing price entirely does not solve adoption. Product design — low friction, pull-based, complete output without ongoing engagement demands — is an equal or greater barrier.

“I tried a free meal prep app, but it felt like homework. Too many notifications.”

— Veronica

Surprises

SURPRISE

Produce waste carries guilt, not just financial frustration

The original framing positioned waste as a financial and logistical failure. Tere’s response revealed an emotional dimension: she feels guilty — she is measuring herself against a standard she believes in and failing to meet it weekly. “Stop feeling guilty every week” and “save money on groceries” are different value propositions aimed at different emotional drivers. The guilt framing may be more motivating for this customer.

SURPRISE

No interviewee mentioned a doctor or healthcare provider as a food resource

All five are pre-diabetic or high risk. Not one mentioned their doctor, a nurse, a dietitian referral, or any healthcare provider in response to where they go for food help. Sources named: TikTok, YouTube, Pinterest, Google, a cookbook, a diabetes app, a meal kit, a mother. Healthcare providers are absent from

the entire data set. The HCP referral channel is a logical opportunity but has zero direct support from these interviews.

SURPRISE

Grace is already trying to solve the problem — and her solution created a new problem

Grace developed a workaround (batch cooking all vegetables at once) and found it failed on texture by day three. She is actively problem-solving and hitting the limits of what an individual can solve without a real system. The product must account for palatability across multiple days, not just planning. Ingredient-first planning solves the buying problem; it does not automatically solve what happens when broccoli is soggy on Thursday.

“I cook them all at once to eat over a couple of days and they get soggy.”

— Grace

SECTION 04

Assumption Scorecard

Five assumptions held going into discovery. Evidence drawn from five interviews, competitive analysis, and systems analysis. Verdicts reflect what the evidence actually supports — not what the venture needs to be true.

Assumption	Evidence For	Evidence Against	Verdict	What to Change
The market is saturated with pre-diabetic tools	PlateJoy raised \$12.3M and built a CDC-recognized diabetes prevention program. Klinio and eMeals are findable through research.	Gap analysis: “no product on the market built specifically for pre-diabetics as a primary consumer customer.” Grace tried a	Disproven	The direct-to-consumer space is largely empty. The absence of competition is not an open lane — it signals that serving this customer well is genuinely hard. Stop treating

Assumption	Evidence For	Evidence Against	Verdict	What to Change
		diabetes app and found it irrelevant. Zero active users of a pre-diabetic product across five interviews.		saturation as the primary risk.
Pricing is a secondary differentiator	None. No interviewee indicated price was not a concern.	Nelsy canceled a meal kit on price. Grace operates on a \$60 hard grocery ceiling. Mary overspends without a plan. Competitive analysis: \$5–15/month is a structural barrier before grocery costs. PlateJoy's B2B pivot was partly driven by the impossibility of charging the consumer directly.	Disproven	Price is a gate, not a differentiator. It must be a primary design constraint from day one. The business model question — who actually pays — is unresolved and is a real problem, not a detail to decide later.
Target customer understands basic blood sugar logic	Tere: "I know the rice isn't great for my sugar." Grace: "I know ramen is bad for sugar." Mary tried a low-	Mary: "I'm not sure about the sugar" in oatmeal. Mary ate Mac	Partially Confirmed	The claim "the problem is not a lack of nutritional knowledge" is too strong. Retire it. Knowledge that

Assumption	Evidence For	Evidence Against	Verdict	What to Change
	carb cookbook, indicating directional awareness.	and Cheese with no blood sugar flag. Grace bought bread and rice on the same trip as ramen with no flag. Veronica shops by “seemed healthy” with no blood sugar framework. None mentioned glycemic index, carb counting, fiber, or meal timing.		exists is surface-level and inconsistent. The product cannot assume customers will evaluate its recommendations correctly. Brief rationale must accompany recommendations so customers can spot-check them.

Assumption	Evidence For	Evidence Against	Verdict	What to Change
The core problem is a system breakdown, not a knowledge gap	Tere: “I have the intention, but not a good system.” All five confirmed produce waste, unplanned shopping, and tool abandonment. Tere and Grace both know specific bad foods and eat them anyway — knowledge present, system fails regardless.	Mary does not know whether oatmeal is safe — that is a knowledge gap, not a system gap. Veronica has no blood sugar framework at all. Grace believes walking compensates for ramen. These beliefs persist independent of what planning system the customer uses.	Partially Confirmed	System gap is primary and confirmed. Knowledge gap is secondary and real — not absent. The product does not need to become a nutrition course, but it cannot be built as though the knowledge gap does not exist. Design for the customer who has partial knowledge and will spot-check outputs.
Pre-diabetic parents would trust an AI system for food planning	Tere wants guidance but “not something time-consuming.” Mary wants something simpler than a cookbook. Grace wants a “trusted, filtered, personal recommendation.” General openness to a better solution exists.	Grace: diabetes app “felt like it was made for older people.” Nelsy trusts her mother over apps. Veronica abandoned a free app for being homework. Every tool tried was	Inconclusive	This assumption was never tested. It must be tested before significant product investment. Suggested question: “If something built your weekly plan around your actual budget, schedule, and blood sugar — and got it right — would it matter

Assumption	Evidence For	Evidence Against	Verdict	What to Change
		abandoned. The baseline is skepticism toward digital tools, not openness. No interviewee was asked about AI directly.		whether a person or an AI built it?” The product depends on a “yes” that has not been obtained.

SECTION 05

Business Model Canvas

☐ Interview-supported

☒ Weak evidence — flagged

KEY PARTNERS △ Required (all paths) - Clinical advisor (RD or CDE) - AI inference provider - Grocery price data source Contingent - Regional grocery chain (Remix 3) - Community health clinic	KEY ACTIVITIES - Weekly plan generation - Glycemic logic curation & validation - Grocery price data integration - Low-friction onboarding - Clinical accuracy QA KEY RESOURCES	VALUE PROPOSITIONS - Eliminates weekly produce waste - Converts inspiration to execution - Removes the weekly restart - Handles blood sugar logic without complete nutritional knowledge - Works for the whole household	CUSTOMER RELATIONSHIPS - Pull-based, not push-based - Complete output, not guidance - Trusted, not clinical - Familiarity over novelty - No ongoing engagement demands CHANNELS - TikTok / YouTube	CUSTOMER SEGMENTS Primary - Pre-diabetic adult, 35+ - Working + caregiving ~\$60 grocery ceiling - Partial blood sugar awareness - Has tried and abandoned tools △ Payer identity unresolved
---	---	---	--	---

/ employer (Remix 2)	- Validated glycemic logic library - AI inference capability - Clinical advisor relationship - Grocery price data - Founder’s lived experience	“I have the intention, but not a good system.” — Tere	(awareness) - SMS / WhatsApp (delivery) - Grocery chain app (Remix 3) - HCP referral (inference only) △ SMS delivery untested
COST STRUCTURE △ - AI inference (variable, scales with usage) - Clinical advisor / glycemic validation - Customer acquisition (channel-dependent) - Legal & liability exposure No real figures available. Liability unscoped. Clinical validation uncosted.		REVENUE STREAMS △ Option 1: Direct subscription (<\$5/mo hypothesis) Option 2: Institutional B2B (\$30–50/member/mo) Option 3: Grocery chain license fee No WTP tested. No price point confirmed.	

Block-by-Block Evidence Assessment

Block	Evidence	Key content	Open gap
Customer Segments	Moderate	Pre-diabetic adult, 35+, working, caregiving, ~\$60 grocery ceiling, partial blood sugar awareness, has tried and abandoned at least one tool. Evidence base: Tere, Grace, Mary (3 interviews).	Payer identity unresolved. If customer cannot pay, she is the user, not the customer — and the segment definition changes.
Value Propositions	Strong	5 value statements, each traceable to specific interview quotes: waste elimination, inspiration-to-execution bridge, removes weekly restart, handles blood sugar logic, works for the whole household.	AI trust not validated. Value cannot be delivered if customer will not follow a plan built by an AI.

Block	Evidence	Key content	Open gap
Channels	Moderate	TikTok/YouTube for awareness (3 of 5 interviewees). SMS/WhatsApp for low-friction delivery (inferred). Grocery chain app is structurally strongest (solves payer + channel + friction simultaneously).	SMS/WhatsApp delivery untested with target customer. HCP referral has no interview support — inference only.
Customer Relationships	Strong	Pull-based not push-based (Veronica: “homework, too many notifications”). Complete output not guidance (Nelsy: mom’s recipes not cooked). Trusted not clinical (Grace: app for older people). No ongoing engagement demands.	AI trust question lives here. What relationship model builds trust for a medically sensitive AI recommendation? Untested.
Revenue Streams WEAK	Weak	Three paths: direct subscription, institutional B2B, grocery chain license. No pricing evidence from interviews — no interviewee was asked what they would pay. Nelsy abandoned a meal kit on price; Veronica abandoned a free app on friction.	Willingness to pay completely untested. No price point confirmed. Must be tested before revenue model decision is made.
Cost Structure WEAK	Weak	Categories identified: AI inference, clinical validation, customer acquisition, legal/liability. All are material. None are costed. Cost structure follows revenue model — which is unresolved.	Liability exposure from incorrect glycemic guidance is unscoped. Clinical validation is uncoded. Both are non-optional at launch.
Key Partners WEAK	Weak	Required regardless of path: clinical advisor, AI provider, grocery price data. Contingent: grocery chain (Remix 3) or clinic/employer (Remix 2). All are logical inferences. None are validated by interview evidence.	Clinical advisor unidentified. Grocery data source unchosen. No institutional partner relationship exists or is in development.
Key Activities	Moderate	Core: plan generation, glycemic logic curation, grocery data integration, low-friction onboarding, clinical QA. Go-to-market: content creation (D2C path) or B2B partnership	B2B pathway undefined. WTP testing not yet run. These are the highest-priority near-term activities

Block	Evidence	Key content	Open gap
		development (institutional/grocery path).	before further product investment.
Key Resources	Moderate	Validated glycemic logic library (primary defensible asset), AI inference capability, clinical advisor, grocery price data, founder's lived experience as member of target segment.	Glycemic logic library does not yet exist. Clinical advisor not yet engaged. These are prerequisites for launch, not post-launch investments.

SECTION 06

Remaining Risks and Next Steps

Risk 1 — Primary: AI trust for a medically sensitive recommendation (untested)

No interviewee was asked about AI. The entire product depends on this. What the interviews do show is that trust in digital tools generally is low: every tool tried was abandoned, and the most trusted source named across all five interviews was a person (Nelsy's mother). Whether AI changes the trust calculus — because the plan is personalized — or makes it worse — because it is opaque and making health recommendations — is unknown.

Validation question: “If something built your weekly grocery plan around your actual budget, your schedule, and what's safe for your blood sugar — and it got it right — would it matter to you whether a person or an AI built it? What would change about your answer?”

Risk 2: Willingness to pay is completely unvalidated

Not one of the five interviewees was asked what they would pay. Budget constraints are confirmed. A specific price point is not. The revenue model cannot be chosen with confidence until this is tested directly.

Risk 3: Behavior change before the crisis point

The system removes the decision only if the customer follows the plan in advance. Tere has to shop on Sunday, buy what the plan says, and have better food in the house before Tuesday night arrives. That is not zero behavior change — it is behavior change shifted earlier in the week. The interviews confirm intent. They do not confirm that any of these five people will build a new habit around a weekly planning tool when they have already abandoned every structured resource they have tried.

Risk 4: Liability from incorrect glycemic guidance

The product generates food recommendations for people managing a diagnosed medical condition. A wrong recommendation has medical consequences. Clinical validation of the glycemic logic library and legal review of the output format and disclaimers are non-optional before launch. Neither has been scoped or costed in any document.

Next Steps by Priority

Priority	Action	Why it unlocks
IMMEDIATE	Return to Tere, Grace, and Mary with AI trust and WTP questions	Tests the single highest-risk assumption (AI trust) and fills the single largest evidence gap (willingness to pay). Both are required before a product decision is defensible.
IMMEDIATE	Recruit 3–5 additional interviews matching the refined profile exactly	Current evidence base is three confirming interviews. That is enough to continue, not enough to build on. Must recruit diagnosed pre-diabetic, working, caregiving, budget-constrained — not adjacent contacts.
NEAR-TERM	Identify and engage a clinical advisor (RD or CDE)	Required before any product touches a user with a diagnosed condition. Also provides the credibility signal needed for the AI trust problem.
NEAR-TERM	Resolve the payer question: direct consumer, institutional, or grocery chain	Every downstream canvas block — Revenue Streams, Cost Structure, Key Partners, Channels — depends on this decision. It cannot be deferred indefinitely without building the wrong thing.
NEAR-TERM	Scope legal and liability exposure	Liability from incorrect glycemic guidance is a real and non-optional cost. It does not appear in any

Priority	Action	Why it unlocks
		document. It needs to be understood before the product is shown to users with a real diagnosis.

BUS 395: Venture Creation with AI — Rollins College — Summer 2026 — Fanny Polanco

Generated from: VENTURE.md • venture_direction_decision.md • interviews/synthesis.md • interviews/patterns.md • assignments/assumption_scorecard.md • assignments/business_model_canvas.md